

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

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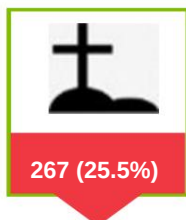
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°38/MVB_21/06/2026

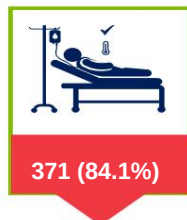
Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (34)	• Ituri (22/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia and Fataki. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 21, 2026
Publication date	June 22, 2026



Total confirmed cases for the 3 provinces



Cumulative deaths among confirmed cases and case fatality rate



Patients in isolation - hospitalization



Cumulative number of recoveries



Suspected cases of the day



Contact tracing rates for the 3 provinces

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

Suspicious deaths occurring in the community are subject to investigation. further investigation with a view to possible reclassification (Non-cases, probable cases).

1. HIGHLIGHTS

- **Forty-five (45) new confirmed cases, including 13 deaths**, were reported at the end of the day on June 21, 2026, in the provinces of **Ituri (38, including 11 deaths) and North Kivu (7, including 2 deaths)**. These confirmed cases are distributed among the following health zones : **Bunia (10), Mongbwalu (9), Nyankunde (7), Rwampara (4), Mangala (4), Tchomia (2), Lita (1), and Nia-Nia (1)** in Ituri province, and **Katwa (3), Beni (1), Butembo (1), Kyondo (1), and Musienene (1)** in North Kivu province. • **Twelve (12) patients** with Ebola virus disease were declared recovered after receiving treatment.
- negative control tests including **9 in Ituri and 3 in North Kivu**.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

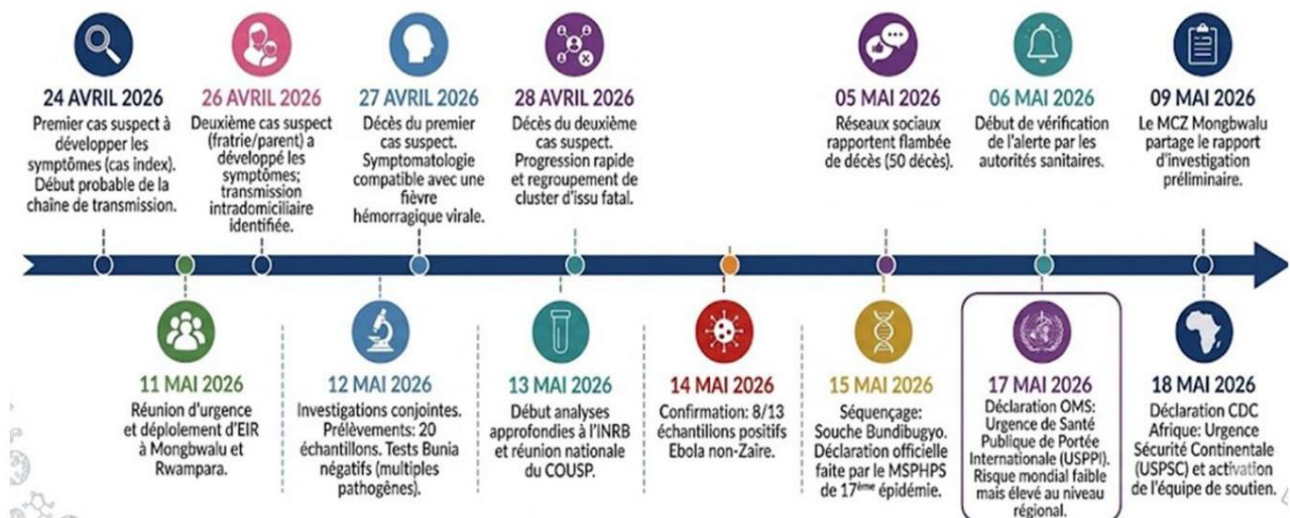


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 21, 2014

Table 1. Distribution of confirmed cases and deaths by affected province as of June 21, 2014.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	954	216	22.6%	22 out of 36 (61.1%)	38
North Kivu	91	50	54.9%	11 out of 34 (32.3%)	7
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1048	267	25.5%	34 out of 104 (32.7%)	45

No new health zones were affected as of June 21, 2014. The number of affected health zones remains at 34/104 (section 3.1). Since May 26, 2014, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission. However, active surveillance remains necessary.

3.2 Distribution by health zone (Cumulative total as of 21/06/2014)

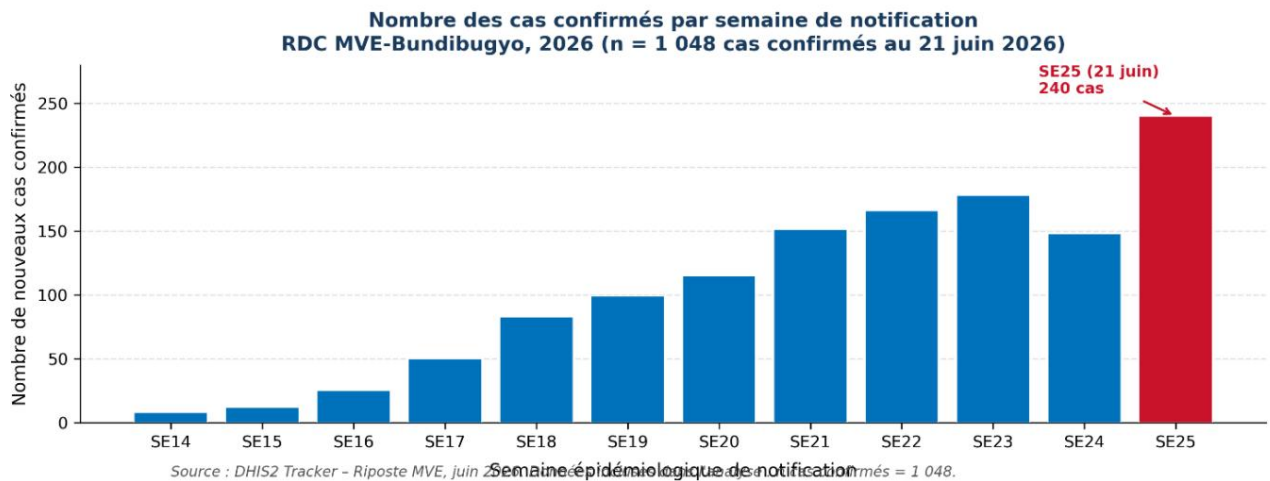
- **Ituri Province (954 cases):** Bunia (285), Rwampara (215), Mongbwalu (229), Nyankunde (86), Nizi (26), Lita (21), Mangala (11), Bambu (14), Komanda (10), Kilo (7), Tchomia (7), Damas (4), Aungba (6), Rimba (3), Aru (3), Mambasa (2), Nian-Nia (2), Logo (2), Drodoro (1), Gety (1), Kambala (1), and Fataki (1).

Seventeen (17) additional cases are awaiting precise breakdown by health zone in the province.

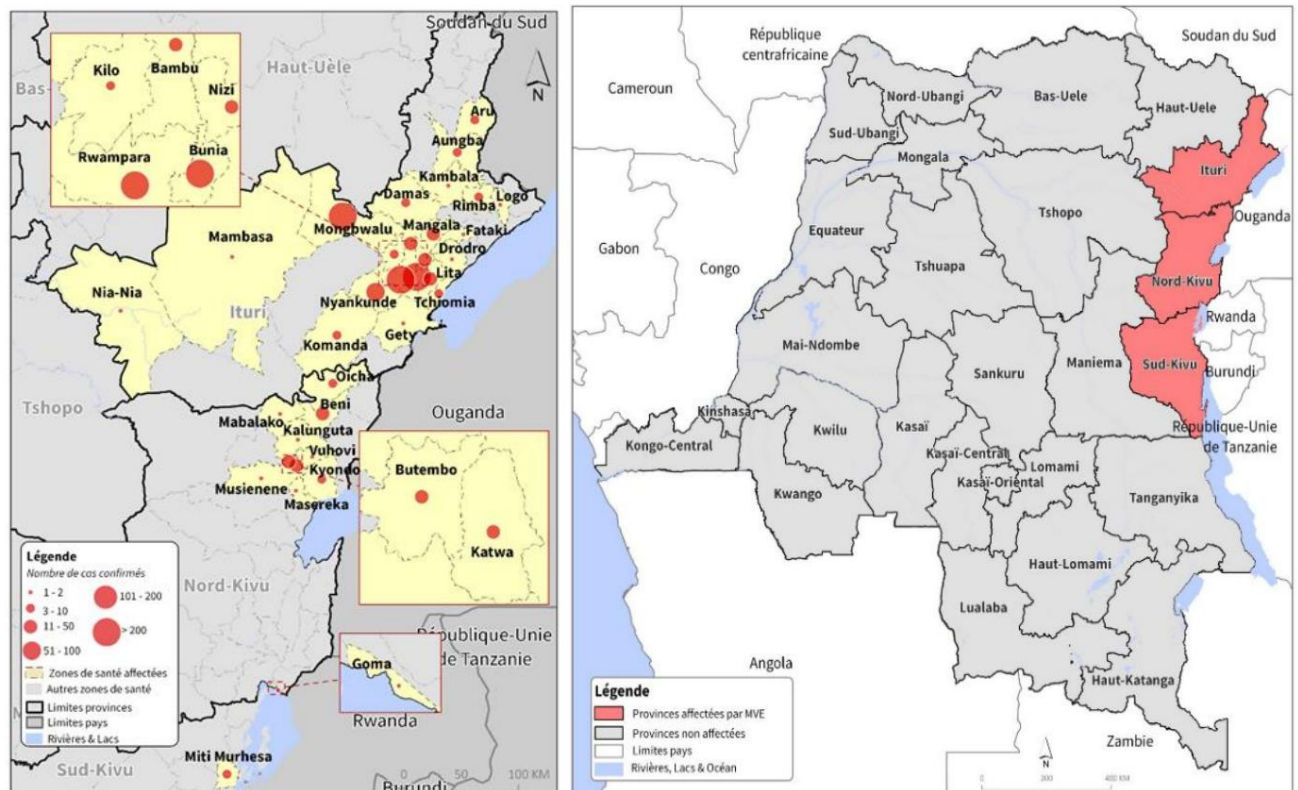
- **North Kivu Province (91 cases):** Katwa (31), Butembo (28), Beni (18), Oicha (3), Kyondo (3), Kalunguta (2), Musienene (2), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).

- **South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time analysis: We observe an increasing number of confirmed cases from one week to the next, reflecting continued transmission of the disease in the community. Rapid geographical expansion of the epidemic remains possible if public health measures are not implemented quickly.



3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of June 21, 2026

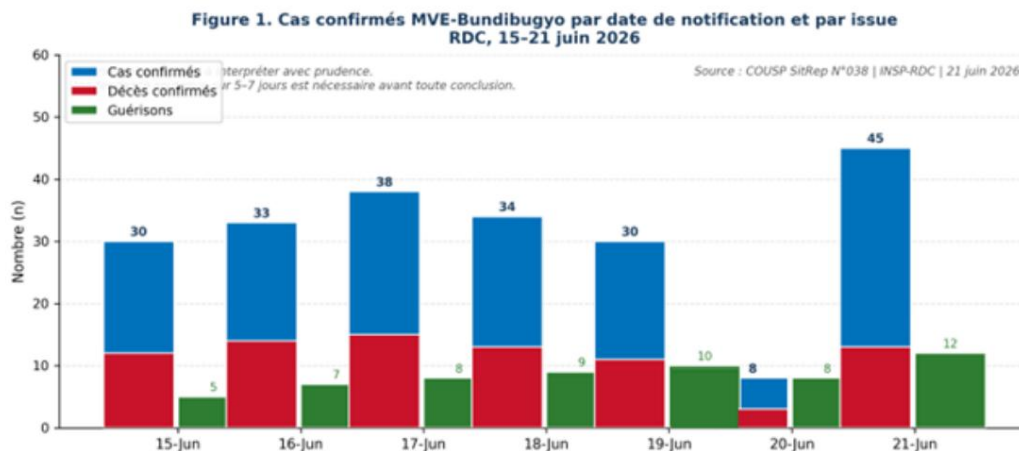


Figure 1. Confirmed MVE-Bundibugyo cases by notification date and outcome — DRC, 15-21 June 2026

⚠ Daily variation should be interpreted with caution. Source: COUSP SitRep N°038 | INSP.

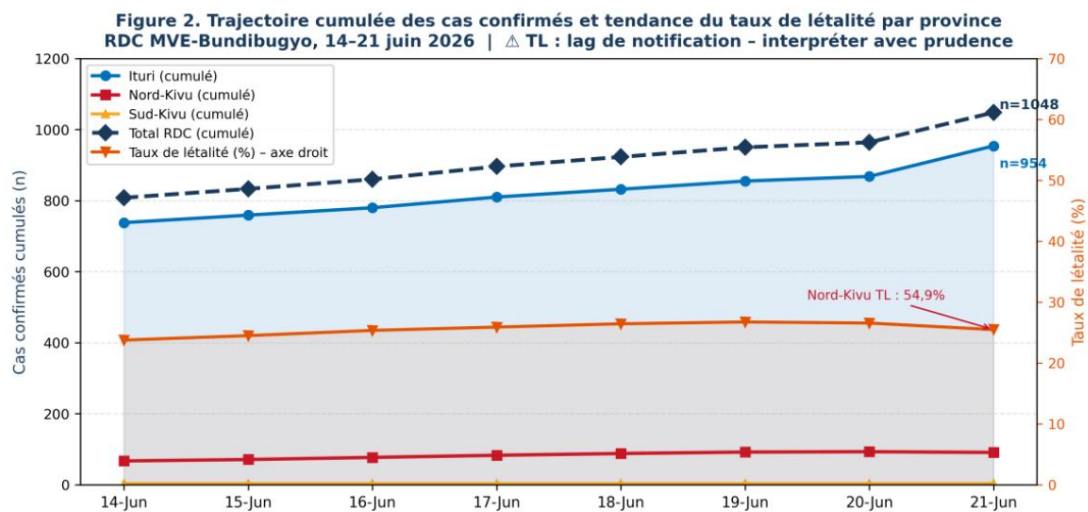


Figure 2. Cumulative trajectory of confirmed cases and trend in the case fatality rate by province — DRC, June 14-21, 2026

Note: The TL calculated on cumulative cases is subject to a notification delay. Source: COUSP SitReps No. 031–No. 038.

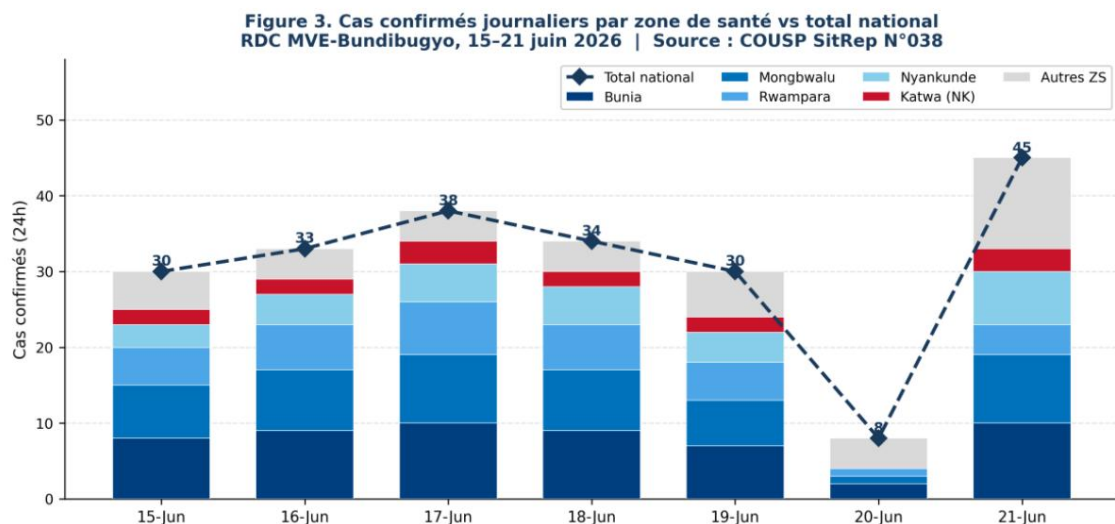


Figure 3. Daily confirmed cases by health zone vs. national total — DRC, June 15-21, 2026

Coloured bars = priority health zones | Dotted line = national total | Source: COUSP SitRep N°038 / INSP.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (June 21, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Lethality (CFR, %)
ITURI	954	216	22.6%
Bunia	285	45	15.8%
Rwampara	215	36	16.7%
Mongbwalu	229	96	41.9%
Nyankunde	86	9	10.5%
Nizi	26	5	19.2%
Lita	21	6	28.6%
Komanda	10	3	30.0%
Bamboo	14	2	14.3%
Kilo	7	1	14.3%
Mangala	11	5	45.5%
Tchomia	7	0	0.0%
Damascus	4	0	0.0%
Aungba	6	2	33.3%
Rimba	3	0	0.0%
Aru	3		33.3%
Mambasa	2		50.0%
Logo	2	1	0.0%
Drodoro	1	1 0 1	100.0%
Gety		0	0.0%
Kambala			100.0%
Nia-Nia	1	1	100.0%
Fataki	1 2 1	2 0	0.0%
Other areas not yet identified	17	0	0.0%
NORTH KIVU	91	50	54.9%
Butembo	28	13	46.4%
Katwa	31	19	61.3%
Blessed	18	12	66.7%
Oicha	3	2	66.7%
Kalunguta	2		50.0%
Kyondo	3		33.3%
Goma		1	0.0%
Masereka		1	0.0%
Vuhovi			100.0%
Mabalako		0	0.0%
Musienene	1		50.0%
SOUTH KIVU	1 1 1 2 3	0 1 0 1 1	33.3%
Miti-Murhesa			33.3%
TOTAL	3 1048	1,267	25.5%

The epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo is progressing, with **45 new confirmed cases including 13 deaths** reported in the last 24 hours, bringing the total to **1,048 confirmed cases** and **267 deaths**, representing an overall case fatality rate of **25.5%**.

Provincial dynamics and case concentration

Approximately **91.0%** of cases (n=954) and **80.9%** of deaths (n=216), representing a case fatality rate of **22.6%**, are concentrated in **Ituri Province**, making it the absolute epicenter of the epidemic. Furthermore, slightly more than three-quarters of the reported cases (n=729) in Ituri Province are located in the most affected health zones, namely Bunia (285 cases), Mongbwalu (229 cases), and Rwampara (215 cases). The Bunia and Rwampara health zones have moderate case fatality rates (15.8% and 16.7%, respectively), while the **Mongbwalu** zone...

The outbreak exhibits marked severity, with a case fatality rate of **41.9%** (96 deaths out of 229 cases), suggesting persistent challenges in early intervention and access to care. Furthermore, 17 additional cases have been reported in previously unidentified health zones, without any associated deaths, which could indicate a geographical expansion requiring further investigation.

Severity of the epidemic in North Kivu

A total of **91 confirmed cases and 50 deaths** have been reported in this province, representing a case fatality rate of 54.9%. This province presents the most concerning profile in terms of severity, higher than the national average. This high case fatality rate is concentrated in the health zones of **Beni** (66.7%), **Katwa** (61.3%), and **Oicha** (66.7%). The Katwa health zone had the highest number of reported cases (n=31), followed by the Butembo health zone (n=28) with a lower case fatality rate (46.4%). A 100% case fatality rate was observed only in the Vuhovi zone (with a small sample size), highlighting the danger posed by the virus in this region and the need for an immediate clinical response.

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Continuation of coordination meetings and support for the pillars of the response.

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of June 21, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		300	518	13	831
Alerts investigated		300	518	13	831
Investigation rate		100.0%	100.0%	100.0%	100.0%
Alerts confirmed	Alive	117	22	3	142
	Deceased	43	17	0	60
Total suspected cases for today		160	39	3	202

For the day of June 21, 2026, **831 alerts** were reported by the three provinces, **831 (100.0%)** were investigated, **202 suspected cases including 60 deaths** were identified (Table 3).

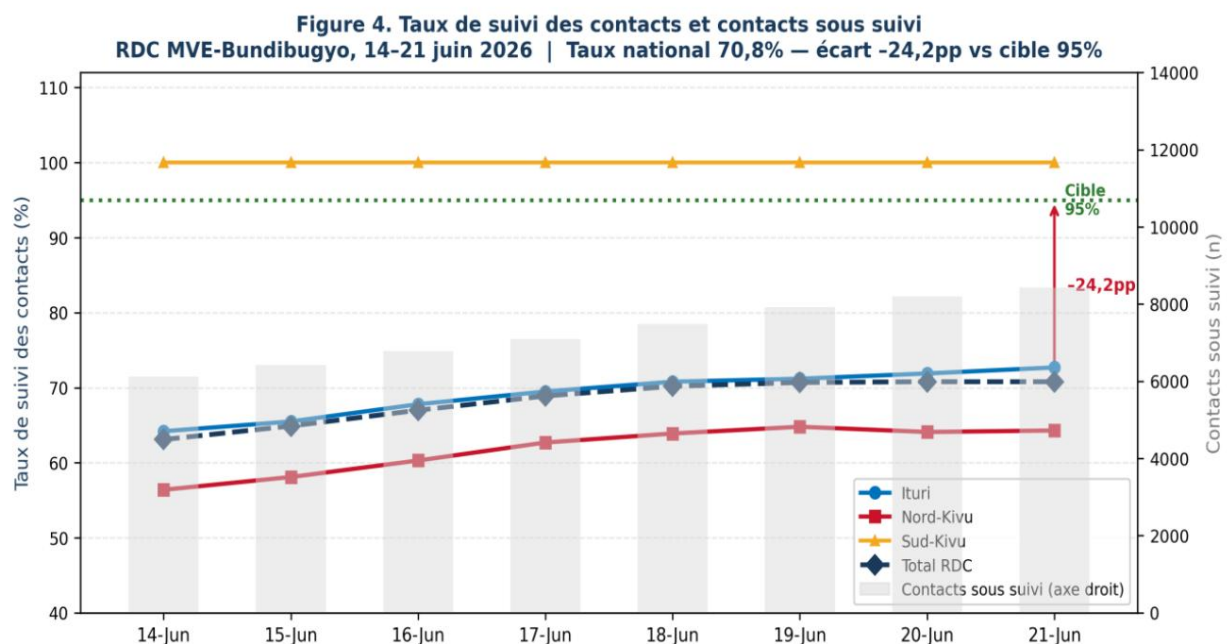
Table 4. Contact tracing of confirmed cases as of June 21, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	6,244*	4,540	72.7%
North Kivu	2,122	1,364	64.3%
South Kivu	58	58	100.0%
Total	8,424	5,962	70.8%

*The follow-up rate in Ituri province was calculated based on the health zones that reported contact follow-up data (completeness: 14/18: 77.7%) •

Continued in-depth investigations and exhaustive listing and systematic follow-up of contacts around confirmed cases in the health zones (narratives of confirmed cases, number of contacts listed and followed per case).

- **653 new contacts** were listed, including **405** in 6 health zones in Ituri province and **248** in the North Kivu province.

**Figure 4. Contact follow-up rate and contacts under follow-up — DRC, June 14–21, 2026**

Green dotted line = target 95%. National deviation: -24.2 pp. Source: COUSP SitReps N°031–N°038.

Update on checkpoint and point of entry (PoC/PoE) activities

- **In North Kivu** : Two (2) live alerts were reported to the Kyavinyonge and Mavivi Barrier PoCs, one (1) of which was confirmed and one (1) invalidated after investigation. The suspected case was referred to the Kyavinyonge CSR in the Kyondo Health Zone.
- **In Ituri** : Six (6) alerts were notified, including 3 live alerts notified to PoC Mudzibala, 2 live alerts notified to PoC Nyamusasi and Foner Mambasa (all were invalidated) and 1 lifeless body intercepted at PoC Pluto released after verification of documents.

Table 5. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of June 21, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	83.6% (46/55)	92.3% (12/13)	85.3% (58/68)
Number of people who went through PoE/PoC	72,826	39,229	112,055
testing, % of travelers	97.0%	96.9%	97.0%
screened, % of travelers who washed their	97.0%	96.6%	97.0%
hands, % of travelers made aware of the issue	97.0%	96.9%	97.0%
Number of alerts notified	6	2	8
Number of validated live alerts	4	1	5
Number of lifeless bodies intercepted today	1	0	1
Number of problematic contacts intercepted today %	0	0	0
of validated live alerts referred to CT/CTE % of	100.0%	100.0%	100.0%
dead bodies swabed	0.0%	0.0%	0.0%
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri:** 243 samples collected and analyzed (positivity 15.6%; n=32).
- **North Kivu :** 91 new samples collected and analyzed (positivity 7.7%; n=7). A total of 52 samples are currently being analyzed in Beni (4) and Butembo (48).
- **South Kivu :** 3 new samples were collected and all came back negative for Ebola virus disease after analysis.

4.4. Infection Prevention and Control (IPC)

Ituri: Completion of 23/25 safe and dignified burials (SDB) in the health zones of Bunia, Rwampara and Mongbwalu; two gaps were reported in Mongbwalu due to resistance from families; decontamination of 6 households and 5 markets around cases in the health zone of Bunia as well as 2 households in the health zone of Rwampara; briefing and awareness-raising on infection prevention and control (IPC) of 172 service providers and community actors, particularly in the health zones of Bunia, Bambu, Rwampara, Komanda and Mambasa.

North Kivu : Evaluation of 8 ESS Scorecards using IPC (average IPC score: 53.9%) followed by a briefing of 62 healthcare providers; completion of 5 EDS in 24 hours; 8 swabs carried out on death alerts reported by the morgue; briefing of 123 healthcare providers from the health zones of Beni, Butembo, Goma and Karisimbi on various IPC themes in the context of the Ebola epidemic; provision of handwashing devices to 8 State Examination Centers in Butembo; decontamination of 2 ESSs (Jerusalem Hospital & Mutiri Hospital) following the transfer of a confirmed positive case to the Ebola Treatment Center in Katwa; PCI scorecard evaluation in 12 ESS, including 2 in the Katwa health zone (CH EERA: 42.5% and MTA OURAGAN 17.5% in the KIHINGA health area) and 10 in the Karisimbi health zone (average score: 50.9%); PCI support for 46 priority ESS in the health zones of Beni, Butembo, Katwa, Goma and Karisimbi.

4.5 Holistic care

- **Twelve (12) patients** with Ebola virus disease have been declared cured after obtaining negative control tests, including 9 in Ituri and 3 in North Kivu.

- Continued nutritional monitoring of patients in CTEs: 796 hot meals were served, including 106 to confirmed cases, 156 to suspected cases, 463 to PPLs and 71 to accompanying persons.

Table 6. Patient movement within healthcare facilities (CTE/CT/CI) as of June 21, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	294	59	7	360
Total admissions (24h)	60	7	3	70
Exits (24h)				
Deceased (Suspects/Confessions)	14	0	0	14
No case	28	0	0	28
Cured	9	3	0	12
Escapees (Suspects/Confessions)	4	0	0	4
Transferred to the HGR	1	0	0	1
Total outflows	56	3	0	59
Patients in isolation (End of Day),	298	63	10	371
including confirmed cases	169	27	0	196
(NC+AC) and suspected cases	129	36	10	175
Overall occupancy rate	88.2%	88.7%	31.3%	84.1%

Mental health activities and psychosocial support

Table 7. Key indicators of mental health and psychosocial support activities as of June 21, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	3 (5.8%)	2 (100.0%)	0
Confirmed cases currently being monitored that have received interventions SMSPS	30 (20.4%)	7 (100.0%)	0
New suspected cases that benefited from SMSPS interventions	7 (9.9%)	2 (100.0%)	0
Suspected cases under follow-up that have received interventions SMSPS	47 (36.4%)	7 (100.0%)	6 (100.0%)
Laboratory results have been announced.	4	3	0
Including positive ones	2	0	0
Number of children separated in daycare who benefited from interventions SMSPS	0	0	1
Number of children separated in the community who benefited from SMSPS interventions	7	1	41
Number of accompanying persons at the CTE who benefited from interventions SMSPS	42	11	17
Number of PPLs who benefited from SMSPS interventions	15	6	12
Number of household and community members who benefited from SMSPS interventions	16	25	7
Number of EDS supported	0	0	0

4.6 Continuity of care

- Ongoing training of 40 midwives and nurses from the Nyankunde and Rwampara health zones in sexual and reproductive health in the context of Ebola.

4.7 Risk communication and community engagement

Ituri: Mass awareness campaign of 3853 people on compliance with Ebola prevention measures, care, community alert reporting and EDS in the health zones of Bunia, Aungba, Nyankunde, Bambu, Nizi, Nia-Nia and Rwampara; Dissemination of 18 spots and messages; Ebola prevention measures; Support from partner FHI 360; 32 service providers briefed on community-based surveillance in the Nia-Nia health zone; Training of 40 service providers on risk communication and community engagement; Educational talk of 2084 referrals for 263 households in the Nyankunde health zone.

Support for other pillars : Monitoring: admission of 4 people to the CTE; raising awareness among a family regarding adherence to CTE referral for a patient presenting symptoms (AS LENGABO);

PCI : Support for the Health Education and Social Development (HED) in three families (AS Hoho) and preparation of one family for acceptance into the HED (ZS Rwampara). Result: HEDs completed

North Kivu: 76,786 people sensitized, 26,723 households visited; capacity building of 250 mobilized RECOs and 164 community actors; 35 Muslim leaders briefed and 22 religious leaders engaged; 18 local radio stations mobilized; 3 communication formats produced and disseminated; 2 rapid assessments carried out with 26 people following 6 cases of escape (4 in KAMI and 2 at the Munigi Ebola Treatment Center); 8,371 people sensitized in markets, parking lots, churches, quarantine sites and Ebola Treatment Centers; educational talk for 20 young leaders from different pressure groups and citizen movements in 7 neighborhoods of the Bungulu commune on the prevention of Ebola and their involvement in preventing violence and attacks against the teams and actions of the Ebola response.

4.8. Logistics

- Receipt of two truckloads of kits and delivery of PCI inputs to the EPST for the state review.
- Thirty (30) vehicles and two (2) ambulances were made available to the various pillars for to ensure operational mobility.
- Three ministry vehicles require maintenance; 127 liters of fuel needed for a vehicle.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued briefings for stakeholders, community awareness-raising on exploitation, abuse and sexual harassment, as well as the signing of codes of good conduct.
- **Ituri :** 126 people briefed (59 women and 67 men); radio broadcast in Swahili on reducing EAS risks in the response; community awareness of 66 people.
- **South Kivu :** PSEA awareness-raising of 1,040 people including 203 women, 164 men, 216 girls and 457 boys in the Katana health zone, among them 5 people living with disabilities, 60 pregnant women and 110 breastfeeding women.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity in CTE and saturation in Ituri (84.1%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTE, deploy additional temporary structures
3 Poor performance in contact tracing (70.8% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4. Low reporting of alerts in certain affected zones	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts
5 confirmed cases not localized or unclassified (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct data reconciliation, remove potential duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.). Active verification in the field.
6 Insufficient supply of MEG and medical supplies	Weakening of overall care	Advocacy and urgent supply of essential medicines
7 Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (78 cases) HCW (including 18 deaths, case fatality rate 23.1%)	Urgent supply, strengthening of IPC training
8 Stockpile of unanalyzed samples (backlog) at the laboratory in North Kivu and delayed diagnosis	Delayed confirmation, high lethality (54.9%)	Deploy a laboratory catch-up plan, strengthen diagnostic capacities
9 Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10. Insufficient coordination and low utilization of COUSP frameworks	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, make operational the Provincial COUSP
11. Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12 Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13. Continuity of Essential Services (CEHS) Compromised	Increase in indirect mortality (not Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14. High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

Provision of PCI kits in 3 ESS in the Mongbwalu health zone, Ituri province.



**FOR ALL
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With technical support from the World Health Organization (WHO) and
of Africa CDC